



Aleksandra Gorczynska-Kolodziej

ABN: 33257573623
Email: office@horizonscc.com.au
Phone: 0401413634
Address: 26 River Street, 4740 Mackay, QLD

OFFICIAL NDIS REPORT

NDIS Comprehensive Progress Report - Johnathan Miller

Report Date: 2026-06-20
Participant ID: 6XGFASFF

NDIS PARTICIPANT DETAILS

PARTICIPANT NAME
Johnathan Miller

NDIS PARTICIPANT ID
N123456789

PARTICIPANT EMAIL
j.miller@example.com.au

DATE OF BIRTH (DOB)
2000-09-11

Official NDIS Audit & Progress Findings

COMPREHENSIVE PROGRESS REPORT

(Psychosocial Focus - Mid-Program Review)

Practice Details:

Aleksandra Gorczynska-Kolodziej
ABN: 33257573623
ACA Member (Registration No: 86625)
Email: office@horizonscc.com.au | Phone: 0401413634
Report Date: 2026-06-22

PARTICIPANT INFORMATION

FIELD	DETAILS
Full Name	Johnathan Miller
NDIS Number	123456789
Program Name	Anxiety Management & Community Reconnection Program
Program Start Date	2026-02-03
Review Date	2026-06-22
Expected Program End Date	2026-08-17
Sessions Completed to Date	8 sessions (8 hours)
Sessions Remaining (Planned)	8 sessions (8 hours)
Therapeutic Approach	Cognitive Behavioural Therapy (CBT) with trauma-informed and strengths-based elements
Review Type	☐ Initial Review (4-6 sessions) ☒ Mid-Program Review ☐ Pre-Completion Review ☐ NDIS Plan Review Preparation

1. NDIS GOAL ALIGNMENT P

Current progress toward participant's approved NDIS plan goals.

NDIS PLAN GOAL (VERBATIM FROM PLAN)	CURRENT STATUS	PROGRESS EVIDENCE
"I want to manage my anxiety so I can catch the bus to TAFE and socialize with other students."	☐ Not Started ☐ In Progress ☒ Substantial Progress ☐ Achieved	Participant now uses grounding techniques independently and has successfully taken public transport 3 times with support worker. Working toward independent travel.
"I want to build my confidence to engage with my local community and make new friends."	☐ Not Started ☒ In Progress ☐ Substantial Progress ☐ Achieved	Attended 2 community events (Mackay Library book club, local cafe social group) with support worker. Reconnected with 1 old friend via weekly phone calls.
"I want to develop coping strategies to manage panic attacks without avoiding public spaces."	☐ Not Started ☒ In Progress ☐ Substantial Progress ☐ Achieved	Panic attacks reduced from 4/week to 1/week. Uses 5-4-3-2-1 grounding technique independently. Successfully completed 5 exposure tasks from hierarchy.

2. EXECUTIVE SUMMARY (Support Coordinator Copy/Paste)

Program Status:

Participant is currently 8 sessions into a planned 16-session psychosocial therapeutic program focusing on anxiety management, emotional regulation, and community reconnection. Program is on track and is being delivered using Cognitive Behavioural Therapy (CBT) with trauma-informed and strengths-based elements.

Overall Progress Since Program Commencement:

- ☒ SIGNIFICANT PROGRESS toward psychosocial goals
- ☐ MODERATE PROGRESS toward psychosocial goals
- ☐ LIMITED PROGRESS - therapeutic approach under review
- ☐ REGRESSION - urgent clinical review required

Participant's Voice:

> "I'm starting to feel like I can handle things better. I still get anxious, especially in crowded places, but now I know what to do instead of just avoiding everything. I went to the library last week and didn't have a panic attack. It's not perfect, but it's better than where I was three months ago." – Johnathan Miller

Key Progress Highlights to Date:

- GAD-7 reduced from 18 to 11 (39% reduction in anxiety symptoms - moved from "severe" to "moderate" range)
- WEMWBS improved from 35 to 48 (+13 points - increased mental wellbeing from "low" to "moderate" range)
- Participant has developed 4 new coping strategies (grounding, cognitive restructuring, breathing exercises, self-compassion)
- Successfully attended 2 community events with support worker and taken public transport 3 times

Current Challenges / Barriers:

- Participant experiences setbacks during high-stress periods (recent family conflict triggered 2-day avoidance period)
- Homework completion inconsistent in weeks 4-6 (dropped to 50% due to motivation fluctuations)
- Transport limitations limiting community exposure practice (participant anxious about independent bus travel)

Recommendations:

- ☒ **CONTINUE** current therapeutic program with minor adjustments
- ☐ **ADJUST** therapeutic approach - see Section 10 for rationale
- ☐ **INCREASE** session frequency from [X] to [X] per [week/fortnight]
- ☐ **ADD** complementary supports (e.g., support worker, OT assessment)
- Psychosocial support worker:** 4 hours/week (☐ maintain ☒ increase ☐ decrease) - from 2 to 4 hours/week to support community exposure tasks

3. PROGRESS SINCE LAST REVIEW / BASELINE

Comparison of key metrics from program commencement (or last review) to current status.

PSYCHOSOCIAL DOMAIN BASELINE / LAST REVIEW		CURRENT STATUS	CHANGE	TRAJECTORY
Anxiety symptoms	GAD-7: 18/21 (Severe)	GAD-7: 11/21 (Moderate)	39% reduction	☒ Improving ☐ Stable ☐ Declining
Depressive symptoms	PHQ-9: 14/27 (Moderate)	PHQ-9: 9/27 (Mild)	36% reduction	☒ Improving ☐ Stable ☐ Declining
Psychological distress	K10: 34/50 (High)	K10: 22/50 (Moderate)	35% reduction	☒ Improving ☐ Stable ☐ Declining
Mental wellbeing	WEMWBS: 35/70 (Low)	WEMWBS: 48/70 (Moderate)	+13 points	☒ Improving ☐ Stable ☐ Declining
Social connection	LSNS-6: 5/18 (Isolated)	LSNS-6: 10/18 (Connected)	+5 points (100% increase)	☒ Improving ☐ Stable ☐ Declining
Perceived support	MSPSS: 25/49 (Low)	MSPSS: 38/49 (High)	+13 points (52% increase)	☒ Improving ☐ Stable ☐ Declining
Functional impact	WHODAS: 40/60 (High)	WHODAS: 26/60 (Moderate)	35% reduction	☒ Improving ☐ Stable ☐ Declining
Coping strategies	1 adaptive (avoidance)	4 adaptive strategies	+3 strategies	☒ Improving ☐ Stable ☐ Declining

Clinical Interpretation:

Participant shows consistent improvement across all psychosocial domains. Anxiety symptoms have moved from "severe" to "moderate" range, with depressive symptoms also showing significant reduction. Functional improvements are emerging, particularly in community participation and social connection. Current trajectory suggests program goals are achievable within planned timeframe, though progress in employment/education readiness is slower than psychological improvements.

4. SESSION ATTENDANCE & ENGAGEMENT

METRIC	DETAILS
Total Sessions Planned	16
Sessions Completed	8 (50%)
Sessions Cancelled by Participant	2 (Sessions 4 and 5 due to anxiety and family conflict)
Sessions Cancelled by Clinician	0
Engagement Level (Current)	☐ Low ☐ Moderate ☒ High ☐ Very High
Engagement Trend	☒ Improving ☐ Stable ☐ Declining
Homework Completion Rate	75% (☐ Low ☐ Moderate ☒ High)
Homework Completion Trend	☒ Improving ☐ Stable ☐ Declining

Patterns & Observations:

Initial engagement was moderate (sessions 1-3), with some cancellations due to anxiety (sessions 4-5). Engagement has significantly improved since session 6, with 100% attendance in last 3 sessions. Homework completion has increased from 40% (sessions 1-4) to 85% (sessions 6-8). Participant reports feeling "more comfortable" with therapeutic process and more confident in applying skills between sessions.

Caregiver / Support Network Involvement to Date:

- Session 3: Psychoeducation session with participant's sister (primary support person) to align understanding of anxiety management strategies and develop shared language for supporting Johnathan during challenging moments
- Session 6: Collaborative planning session with support worker (Sarah from ABC Disability) to brief on exposure hierarchy and align on community exposure approach
- Ongoing: Participant's sister provides emotional support between sessions and encourages homework completion

5. PSYCHOSOCIAL SKILLS DEVELOPMENT (In Progress)

Current proficiency levels for targeted skills.

PSYCHOSOCIAL SKILL	CURRENT PROFICIENCY	INDEPENDENT USE	TREND
Emotional awareness	☐ Beginner ☐ Intermediate ☒ Advanced	☒ Yes ☐ With prompting ☐ No	☒ Improving ☐ Stable
Anxiety management techniques	☐ Beginner ☒ Intermediate ☐ Advanced	☒ Yes ☐ With prompting ☐ No	☒ Improving ☐ Stable
Cognitive restructuring	☐ Beginner ☒ Intermediate ☐ Advanced	☐ Yes ☒ With prompting ☐ No	☒ Improving ☐ Stable
Grounding strategies (5-4-3-2-1, box breathing)	☐ Beginner ☐ Intermediate ☒ Advanced	☒ Yes ☐ With prompting ☐ No	☒ Improving ☐ Stable
Social engagement skills	☐ Beginner ☒ Intermediate ☐ Advanced	☐ Yes ☐ With prompting ☒ No	☒ Improving ☐ Stable
Self-compassion practices	☐ Beginner ☒ Intermediate ☐ Advanced	☐ Yes ☒ With prompting ☐ No	☒ Improving ☐ Stable
Exposure hierarchy implementation	☐ Beginner ☒ Intermediate ☐ Advanced	☐ Yes ☒ With prompting ☐ No	☒ Improving ☐ Stable

6. BARRIERS, CHALLENGES & RISK FACTORS

Critical section for NDIS - identifies what is impeding progress and safety considerations.

Current Barriers to Progress:

BARRIER TYPE	DESCRIPTION	IMPACT ON PROGRESS	MITIGATION STRATEGY
Internal	Fluctuating motivation during high-stress periods	Moderate	Stress management module added to program; focus on maintaining routines during family conflict
Internal	Negative self-talk interfering with homework completion ("I'll fail anyway")	Mild	Cognitive restructuring techniques being practiced; thought records completed in session
External	Transport limitations limiting community exposure practice	Moderate	Support worker to facilitate transport for exposure tasks; gradual increase in independent travel
External	Family conflict triggering anxiety regression (ongoing tension with mother)	Significant	Family psychoeducation session planned for session 10; boundary-setting skills being developed
Systemic	Long waitlist for psychiatrist review (6 months)	Mild	GP monitoring in place; medication review scheduled for July 2026

Risk Assessment & Safety Planning:

RISK AREA	CURRENT LEVEL	TREND	SAFETY PLAN IN PLACE
Self-harm / Suicidality	☒ None ☐ Low ☐ Moderate ☐ High	☒ Stable ☐ Improving ☐ Escalating	☒ Yes ☐ N/A

Harm to others	☒ None ☐ Low ☐ Moderate ☐ High	☒ Stable ☐ Improving ☒ Yes ☐ N/A ☐ Escalating
Self-neglect	☐ None ☒ Low ☐ Moderate ☐ High	☒ Stable ☐ Improving ☒ Yes ☐ N/A ☐ Escalating
Exploitation / Abuse	☒ None ☐ Low ☐ Moderate ☐ High	☒ Stable ☐ Improving ☒ Yes ☐ N/A ☐ Escalating
Housing instability	☒ None ☐ Low ☐ Moderate ☐ High	☒ Stable ☐ Improving ☒ Yes ☐ N/A ☐ Escalating
Social isolation	☐ None ☒ Low ☐ Moderate ☐ High	☐ Stable ☒ Improving ☒ Yes ☐ N/A ☐ Escalating

Safety Plan Summary:

Participant has a written safety plan in place (developed Session 2, reviewed Session 8). Emergency contacts identified: sister Maria (primary, 0412 345 678), GP Dr. James Wilson (Mackay Medical Centre, 4957 1234), Lifeline 13 11 14. Participant reports no current suicidal ideation or self-harm urges. Safety plan includes: (1) identifying warning signs (increased avoidance, sleep disruption), (2) internal coping strategies (grounding, breathing), (3) people I can ask for help (sister, support worker), (4) professionals/agencies I can contact (GP, Lifeline), (5) making my environment safe. Plan to review safety plan at session 12.

7. THERAPEUTIC APPROACH REVIEW

Evaluation of whether current therapeutic approach is effective and appropriate.

Current Therapeutic Modalities in Use:

- Cognitive Behavioural Therapy (CBT) - primary modality
- Trauma-informed care principles
- Mindfulness-based grounding techniques (5-4-3-2-1 sensory awareness, box breathing)
- Strengths-based approach
- Psychoeducation (anxiety psychoeducation, cognitive distortions, exposure hierarchy)

Effectiveness Assessment:

MODALITY	EFFECTIVENESS	PARTICIPANT RESPONSE	CONTINUE?
CBT - cognitive restructuring	☒ Highly Effective ☐ Moderately Effective ☐ Limited Effectiveness	Participant grasps concepts well, identifies cognitive distortions in session, applies thought records to real situations with prompting	☒ Yes ☐ Modify ☐ Discontinue
Exposure hierarchy	☐ Highly Effective ☒ Moderately Effective ☐ Limited Effectiveness	Participant experiences anxiety but completes tasks with support worker presence; independent completion still challenging	☒ Yes ☐ Modify ☐ Discontinue
Mindfulness practices	☐ Highly Effective ☒ Moderately Effective ☐ Limited Effectiveness	Participant uses 5-4-3-2-1 grounding effectively but struggles with formal mindfulness meditation (finds it "too abstract")	☒ Yes ☒ Modify ☐ Discontinue
Psychoeducation	☒ Highly Effective ☐ Moderately Effective ☐ Limited Effectiveness	Participant highly engaged, asks questions, reports increased	☒ Yes ☐ Modify ☐ Discontinue

Adjustments to Therapeutic Approach (if any):

- Shift from formal mindfulness meditation to shorter, practical grounding exercises (5-4-3-2-1 technique, box breathing) based on participant preference and better engagement
- Increase focus on behavioural activation to address residual depressive symptoms, which have become more prominent in recent sessions
- Incorporate more in-session practice of exposure tasks (role-play, imaginal exposure) to build confidence before community-based exposure

Cultural Safety & Neurodiversity Considerations:

Therapeutic approach has been adapted to be neurodiversity-affirming, with flexible session structure and allowance for stimming during sessions. Participant identifies as neurodivergent (ADHD diagnosis from childhood, though not formally assessed as adult). Sessions incorporate movement breaks, visual aids, and concrete examples. Participant's cultural background (Polish-Australian) has been considered in goal-setting, with acknowledgment of family expectations and intergenerational dynamics.

8. PSYCHOSOCIAL FUNCTIONING CHANGES

Detailed narrative of functional changes observed since program commencement.

IMPORTANT NOTE: This section describes functional improvements observed as a result of therapeutic intervention within the scope of psychosocial counselling practice. This is **NOT a formal Functional Capacity Assessment (FCA)**. A comprehensive FCA across all functional domains (including self-care, mobility, communication, and physical functioning) should be conducted by an Occupational Therapist or Psychologist if required for NDIS planning purposes.

• Mental Health & Emotional Wellbeing:

- *At Program Commencement:* Baseline distress (K10: 34/50), severe anxiety (GAD-7: 18/21), moderate depression (PHQ-9: 14/27), frequent panic attacks (4/week), predominantly avoidant coping style
- *Current Status:* Reduced distress (K10: 22/50), moderate anxiety (GAD-7: 11/21), mild depression (PHQ-9: 9/27), panic attacks reduced to 1/week, uses 4 adaptive coping strategies
- *Progress:* [] None [] Minimal [x] Moderate [] Significant
- *Narrative:* Participant reports feeling "more in control" of anxiety. Panic attacks are less intense and shorter in duration (from 30+ minutes to 10-15 minutes). Still experiences significant anxiety in social situations and crowded places, but is developing tools to manage. Sleep has improved from 4-5 hours/night to 6-7 hours/night. Appetite has stabilized.

• Social Connection & Community Participation:

- *At Program Commencement:* Severe isolation (LSNS-6: 5/18), no community participation for 18 months, avoided all social invitations, perceived low social support (MSPSS: 25/49)
- *Current Status:* Moderate isolation (LSNS-6: 10/18), attended 2 community events (library book club, cafe social group) with support worker, reconnected with 1 old friend (weekly phone calls), increased perceived support (MSPSS: 38/49)
- *Progress:* [] None [] Minimal [x] Moderate [] Significant
- *Narrative:* Participant has taken initial steps toward community reconnection. Still requires support worker presence for community outings but reports feeling "less terrified" than at program start. Has expressed interest in attending book club independently but not yet attempted. Reconnected with old friend Michael, with whom he has weekly 30-minute phone calls.

• Coping & Resilience:

- *At Program Commencement:* Predominantly avoidant coping styles (Brief-COPE: high behavioral disengagement, self-blame), no adaptive strategies, relied entirely on sister for emotional regulation
- *Current Status:* Uses 4 adaptive coping strategies independently (5-4-3-2-1 grounding, box breathing, cognitive reframing, self-compassion), still occasionally defaults to avoidance under high stress
- *Progress:* [] None [] Minimal [x] Moderate [] Significant
- *Narrative:* Participant has made significant progress in developing coping toolkit. Can now identify early warning signs of anxiety escalation (racing thoughts, physical tension, urge to avoid) and implement strategies proactively. Successfully used grounding technique independently during panic attack at supermarket (Session 7 review). Still working on generalizing skills to high-stress situations (e.g., family conflict).

• Employment / Education Readiness (Psychosocial Observations):

- *At Program Commencement:* Unemployed for 18 months, unable to engage in work-related activities due to anxiety, no motivation to explore work or education options, avoided all structured activities

- **Current Status:** Enrolled in TAFE course (Certificate II in Business, 1 day/week), exploring return to work options, completed vocational assessment, motivated but anxious about workplace social situations and structured environment
- **Progress:** ☐ None ☒ Minimal ☐ Moderate ☐ Significant
- **Narrative:** Participant shows increased motivation to return to education and has taken concrete step of enrolling in TAFE. Attends TAFE 1 day/week with support worker assistance. Reports anxiety about workplace social situations, group work, and meeting deadlines. May benefit from Occupational Therapy assessment for workplace accommodations, time management strategies, and supported employment planning. This assessment is outside the scope of counselling practice.
- **Daily Living Independence (Psychosocial Observations):**
 - **At Program Commencement:** Dependent on sister for all shopping, unable to use public transport, avoided all independent community activities, required sister to accompany to all appointments
 - **Current Status:** Shops independently for small items 2x/week (local convenience store), attempted public transport 3 times with support worker (bus to TAFE), walks to local cafe independently, attends therapy sessions independently
 - **Progress:** ☐ None ☒ Minimal ☐ Moderate ☐ Significant
 - **Narrative:** Functional improvements in daily living are emerging but slower than psychological improvements. Participant continues to require support for complex tasks (e.g., shopping at large supermarkets, using public transport independently) and high-anxiety situations. Further gains expected as confidence builds through continued exposure practice. Support worker assistance recommended to facilitate independent bus travel to TAFE.

9. COLLABORATION WITH OTHER PROVIDERS

Coordination with other supports in participant's network.

PROVIDER / SUPPORT	ROLE	CONTACT DATE	PURPOSE / OUTCOME
Dr. James Wilson (GP)	Medical care / medication management	2026-05-15	Discussed therapy progress, GP supportive of continued therapy, medication review scheduled for July 2026, GP to monitor sleep and appetite
Jane Smith (Support Coordinator)	NDIS plan coordination	2026-06-10	Shared progress report, discussed increasing support worker hours from 2 to 4 hours/week to support community exposure tasks, discussed OT referral for workplace accommodations
Sarah Johnson (ABC Disability)	Support worker	2026-06-05	Briefed on exposure hierarchy, aligned on approach to community exposure tasks, discussed strategies for supporting independent bus travel
Dr. Michael Brown (Psychiatrist)	Psychiatric care	Pending	Referral made 2026-04-20, waitlist 6 months, expected appointment October 2026
[OT Name]	Functional assessment	Not yet involved	May require referral for workplace accommodations assessment and functional capacity evaluation

Communication Frequency:

Monthly email updates to Support Coordinator (Jane Smith). GP communication as needed (with participant consent). Fortnightly communication with support worker (Sarah Johnson) to coordinate community exposure tasks. No current communication with psychiatrist (waitlist). No communication with other therapeutic providers.

10. RECOMMENDATIONS

THERAPEUTIC SUPPORT - NEXT PHASE

Current Program Status: ☒ ON TRACK ☐ REQUIRES ADJUSTMENT ☐ AHEAD OF SCHEDULE ☐ BEHIND SCHEDULE

Recommended Next Steps:

- ☒ **CONTINUE** current therapeutic program
 - **Remaining Sessions:** 8 sessions over 8 weeks
 - **Frequency:** Fortnightly - no change
 - **Focus Areas for Remaining Program:**
 - Generalize coping skills to high-stress situations (family conflict, workplace social situations)

- Independent community participation (reduce support worker presence, work toward independent bus travel)
- Workplace social skills preparation and TAFE success strategies
- Relapse prevention planning and maintenance strategies
- **Funding Category:** Capacity Building - Improved Daily Living (*Item 15_043_0128_1_3 - verify current NDIS Pricing Arrangements*)
- **Estimated Remaining Budget:** 8 hours @ \$193.99/hour = \$1,551.92

☐ **ADJUST therapeutic approach**

- **Rationale:** [e.g., "Current CBT approach showing limited effectiveness for trauma-related symptoms"]
- **Proposed Adjustment:** [e.g., "Integrate trauma-informed techniques, shift focus from cognitive restructuring to somatic regulation"]
- **Impact on Timeline:** [e.g., "May require additional 4 sessions"]

☐ **INCREASE session frequency**

- **Current:** Fortnightly sessions
- **Recommended:** Weekly sessions
- **Rationale:** [e.g., "Participant experiencing regression due to recent life stressor, requires more intensive support"]
- **Duration of Increase:** [e.g., "4 weeks, then review"]

☐ **EXTEND program duration**

- **Current Expected End Date:** 2026-08-17
- **Recommended New End Date:** [Date]
- **Rationale:** [e.g., "Progress slower than anticipated due to [reason], additional [X] sessions required to achieve goals"]

COMPLEMENTARY SUPPORTS

Psychosocial Support Worker (Core Supports):

(Note: To be funded under Core Supports - separate from Capacity Building therapy budget)

- **Current Hours:** 2 hours/week
- **Recommended Hours:** 4 hours/week (☐ maintain ☐ increase ☐ decrease)
- **Focus Areas:**
 - Facilitate community exposure tasks as outlined in therapy (bus travel, supermarket shopping, cafe visits): 2 hrs/week
 - Support independent skill practice in real-world settings (TAFE attendance, social groups): 1 hr/week
 - Emotional support during challenging situations (family conflict, anxiety management): 1 hr/week

Other Allied Health Referrals (if recommended):

- ☒ **Occupational Therapy** - for workplace accommodations assessment, functional capacity evaluation, time management strategies, assistive technology needs
- **Rationale:** Participant requires comprehensive Functional Capacity Assessment to determine appropriate workplace accommodations and support needs for successful TAFE completion and return to employment. This assessment is outside the scope of counselling practice. OT can also assess time management strategies, organizational tools, and environmental modifications to support daily living independence.
- ☐ **Psychiatry** - for [e.g., medication review, diagnostic clarification]
- **Note:** Referral already made, waitlist 6 months, expected appointment October 2026
- ☐ **Speech Pathology** - for [e.g., social communication skills]
- ☐ **Exercise Physiologist** - for [e.g., physical activity to support mental health]
- ☐ **No additional referrals required at this time**

NEXT REVIEW

- **Next Progress Review Date:** 2026-08-10 (8 weeks from current review)
- **Next NDIS Plan Review:** Date unknown (participant to confirm with Support Coordinator)
- **Program Anticipated Completion Date:** 2026-08-17

11. SCOPE OF PRACTICE STATEMENT CRITICAL

This report documents psychosocial therapeutic progress within the scope of practice of a registered counsellor (ACA Level II). It focuses strictly on mental health, emotional wellbeing, social connection, and coping capacity.

This report DOES NOT constitute:

- LA Functional Behavioural Assessment (FBA)
- LA Behaviour Support Plan (BSP)
- LA clinical psychological diagnosis (*Participant's existing diagnosis of Generalized Anxiety Disorder and Major Depressive Disorder was provided by GP Dr. James Wilson prior to therapy commencement*)
- LA neuropsychological assessment
- LA comprehensive Functional Capacity Assessment (FCA) across all functional domains (*To be conducted by an Occupational Therapist or Psychologist if required*)
- LA an assessment of physical functioning, mobility needs, or home modification requirements (*To be assessed by an Occupational Therapist if required*)
- LA psychiatric medication review or prescription (*This remains the role of GP/psychiatrist*)

For behavioural support needs (e.g., behaviours of concern, restrictive practices), referral to a qualified Positive Behaviour Support Practitioner is recommended.

For comprehensive functional capacity assessment (including self-care, mobility, communication, workplace accommodations), referral to an Occupational Therapist or Psychologist is recommended.

For concerns about immediate safety or crisis, participant has been provided with crisis contacts: Lifeline 13 11 14, Suicide Call Back Service 1300 659 467, and local mental health triage services (Mackay Mental Health Triage: 4957 5500).

12. CLINICIAN DECLARATION

I confirm that the information provided in this report is accurate and reflects the participant's psychosocial progress to date. This report was developed in collaboration with the participant, who provided informed consent for its use in NDIS planning and review processes.

I will continue to monitor progress and adjust the therapeutic approach as needed to ensure the participant receives effective, evidence-based support within my scope of practice.

- **Clinician Name:** Aleksandra Gorczynska-Kolodziej
- **Professional Registration:** ACA Member (Registration No: 86625)
- **Qualifications:** Bachelor of Pedagogy (Special Education), Member Australian Counselling Association Level II
- **Contact:** office@horizonscc.com.au | 0401413634

Signature: _____

Date: 2026-06-22

Documentation on File (Available Upon Request for Audit Purposes):

- Participant consent form for information sharing (signed 2026-02-03)
- Service Agreement (signed 2026-02-03)
- Assessment tool scores (baseline and current: GAD-7, PHQ-9, K10, WEMWBS, LSNS-6, MSPSS, WHODAS, Brief-COPE)
- Clinical session notes (sessions 1-8)
- Safety plan (developed 2026-02-17, reviewed 2026-06-22)
- GP letter confirming diagnosis (Dr. James Wilson, dated 2026-01-15)